



Department
of Health &
Social Care

Form

Guidance on completing the NHS continuing healthcare consent form

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Applies to England

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Introduction

This guidance is to help you as the responsible professional in completing the NHS continuing healthcare consent form.

Your role is to help ensure that the individual:

- is fully informed
- understands the questions being asked
- is able to make a decision that is in their best interests

This includes:

- explaining the purpose of the form
- assessing whether the individual has the capacity to make the decision
- offering any necessary support or clarification

Read this guidance in conjunction with the [NHS continuing healthcare consent form \(/admin/government/admin/editions/1758872\)](/admin/government/admin/editions/1758872).

Consent is not required from individuals for actions necessary to meet legal requirements relating to the NHS continuing healthcare process. These actions include the carrying out of the checklist or assessment process by health and social care professionals, and the sharing of relevant information between themselves and the care providers involved.

The organisation proposing to share the individual's health and/or care information with friends, family members and representatives must ensure personal data is processed in accordance with data protection laws and relevant regulations. It is best practice for the responsible professional completing the NHS continuing

healthcare checklist to complete the continuing healthcare consent form as that is the first time an individual's information may be shared with friends, family members and representatives.

Integrated care boards (ICBs) and local authorities should have agreed processes in place for completing the NHS continuing healthcare consent form during the referral process for NHS continuing healthcare eligibility.

It is expected that the overall NHS continuing healthcare assessment process should, in most cases, not exceed 28 calendar days from the date that the ICB receives the positive checklist (or, where a checklist is not used, the ICB receives other notice of potential eligibility) to the eligibility decision for continuing healthcare being made.

Note that day 1 of the 28-day timeline (the earliest notification that full continuing healthcare assessment is required) is from the date the ICB receives the checklist, irrespective of whether or not the consent form is received at the same time. Completion of the consent form should not cause undue delay to the continuing healthcare assessment process.

Consent is not required from individuals for actions necessary to meet legal requirements relating to the NHS continuing healthcare process. These actions include the carrying out of the checklist or assessment process by health and social care professionals, and the sharing of relevant information between themselves and the care providers involved where this is necessary for the purpose of direct patient care.

If the individual is deceased

If the individual is deceased, do not complete the form.

If a request for a continuing healthcare checklist or assessment is received from a personal representative or a person who may have a claim arising out of the person's death, see [Access to the health and care records of deceased people \(https://transform.england.nhs.uk/information-governance/guidance/access-to-the-health-and-care-records-of-deceased-people/\)](https://transform.england.nhs.uk/information-governance/guidance/access-to-the-health-and-care-records-of-deceased-people/) for further guidance.

Part 1: consent from individuals who lack capacity to make the decision

If you have reason to believe the person may lack the relevant capacity, first complete the mental capacity assessment at appendix 1 of the form.

If you determine the person lacks capacity, now either complete part 1a (for attorneys or court appointed deputies) or part 1b (for representatives).

Part 1a: to be completed by persons holding a lasting power of attorney or for court appointed deputies

Where an individual lacks capacity to make the decision, consent will need to be sought from any person holding appropriate authority. This could be someone holding a valid and applicable lasting power of attorney (LPA) for the individual or a court appointed deputy (CAD).

Lasting power of attorney

There are 2 types of LPA:

- health and welfare
- property and financial affairs

An LPA must be registered with the Office of the Public Guardian (<https://www.gov.uk/government/organisations/office-of-the-public-guardian>) (OPG). Proof of this status will be required. Decisions regarding consent to sharing health and care information should be taken by a health and welfare attorney only if they have appropriate authority. Some decisions relating to NHS continuing healthcare may be

taken by a property and financial affairs attorney. For more information, see sections 8.7 to 8.9 of the National framework for NHS continuing healthcare and NHS-funded nursing care (<https://www.gov.uk/government/publications/national-framework-for-nhs-continuing-healthcare-and-nhs-funded-nursing-care>).

Court appointed deputy

An individual may have a CAD for personal welfare to act for them, who may have authority to consent to the sharing of information. You should seek evidence of the legal status of the deputy and their authority, and they should share the relevant court order.

You can apply for free to search OPG registers to see if someone has another person acting on their behalf. For more information, see Find out if someone has an attorney, deputy or guardian acting for them (<https://www.gov.uk/find-someones-attorney-deputy-or-guardian>).

A person holding either of these positions is entitled to make decisions about sharing the individual's information with family, friends and representatives, if they have been given authority to make this decision.

Any decision by an attorney or deputy to share the information or not must be made in the person's best interests as set out in the Mental Capacity Act 2005 (<https://www.legislation.gov.uk/ukpga/2005/9/contents>) and Mental Capacity Act Code of Practice (<https://www.gov.uk/government/publications/mental-capacity-act-code-of-practice>).

Part 1b: to be completed where an individual does not have an attorney or deputy

Where an individual does not have an attorney or deputy, then a decision must be taken in the individual's best interests as set out in the Mental Capacity Act 2005. For more information, see Make decisions on behalf of someone: how to make decisions (<https://www.gov.uk/make-decisions-for-someone/making-decisions>).

In all cases, the decision maker must consider, among other matters, any previously stated wishes of the individual regarding information-sharing (made at a time they had capacity) where reasonably ascertainable, and

whether the loss of capacity is temporary or permanent.

Part 2: consent from individuals who have the capacity to make the decision

This section is to obtain and record consent from an individual where they have the capacity to make the decision to share information with family, friends or representatives.

The individual should read the 'statement of consent' carefully. They should then confirm to indicate their agreement.

As the responsible professional you should explain the implications of both consenting and not consenting, helping to ensure that the individual provides informed consent.

As the responsible professional you should also inform the individual that they have the right at any time to:

- change their mind
- withdraw their consent
- amend their consent

If they wish to do this, they should contact a health or care professional involved in their care.

Part 3: declaration by responsible professional

The responsible professional obtaining consent should complete the declaration.



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